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Background: First-line treatment with immunotherapy targeting the PD-(L)1 pathway, ±chemotherapy, has improved outcomes for patients (pts) with a/m NSCLC. Despite this, some pts may not respond to anti-PD-(L)1 treatment or develop resistance, meaning new treatment strategies are needed. Dato-DXd, a TROP2-directed antibody-drug conjugate, has shown promising efficacy and a manageable safety profile in pts with NSCLC, both as monotherapy and with immunotherapy ± chemotherapy. Rilvegostomig (AZD2936), a bispecific antibody that simultaneously inhibits PD-1 and TIGIT on the same T cell, has shown preliminary efficacy and a favourable safety profile in a/m NSCLC. TROPION-Lung10 (NCT06357533) is evaluating the potential of the directed cytotoxicity of Dato-DXd combined with the immune checkpoint inhibition of rilvegostomig to improve outcomes for pts with a/m NSQ NSCLC vs SoC pembrolizumab.

Trial design: TROPION-Lung10 is a phase III, randomised, open-label, global study. Approximately 675 adults with NSQ Stage IIIB/C or IV NSCLC (per AJCC edition 8) not amenable to curative surgery or definitive chemoradiation, no AGAs and high PD-L1 expression (≥50% of tumour cells), who have ECOG PS 0/1, will be enrolled. Pts will be randomised (2:1:2) to Dato-DXd (6 mg/kg IV Q3W) plus rilvegostomig (750 mg IV Q3W), rilvegostomig monotherapy (750 mg IV Q3W) or pembrolizumab monotherapy (200 mg IV Q3W). Treatment will continue until disease progression (per RECIST v1.1), unacceptable toxicity, or until other discontinuation criteria are met. Randomisation will be stratified by smoking status, geographical location and TROP2 biomarker status. For endpoints see table. Dual primary endpoints will be estimated using the Kaplan-Meier method and analysed using a stratified log-rank test. The first pt was enrolled in April 2024; enrolment is ongoing.

Table: 122TiP

Study endpoints		Population	
		TROP2+	Full analysis set
Dual primary	PFS by BICR	a	
	OS	a	
Secondary	PFS by BICR	b, c	a, b, c
	OS	b, c	a, b, c
	Objective response rate by BICR	a, b, c	a, b, c
	Duration of response	a, b, c	a, b, c
	PFS2	a	a
	Patient-reported outcomes	a	a
	Pharmacokinetics: Dato-DXd + rilvegostomig & rilvegostomig		
	Immunogenicity: Dato-DXd + rilvegostomig & rilvegostomig Safety/tolerability ^{a,b}		

^aDato-DXd + rilvegostomig vs pembrolizumab. ^bRilvegostomig vs pembrolizumab.

^cDato-DXd + rilvegostomig vs rilvegostomig.

BICR, blinded independent central review; OS, overall survival; PFS, progression-free survival; PFS2, second progression-free survival; TROP2, trophoblast cell surface antigen 2.

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Legal entity responsible for the study: AstraZeneca.

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TROPION-Lung14: A phase III study of osimertinib ± datopotamab deruxtecan (Dato-DXd) as first-line (1L) treatment for patients with EGFR-mutated locally advanced or metastatic (LA/M) non-small cell lung cancer (NSCLC)

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Background: Despite the benefit observed with osimertinib, most patients with LA/M EGFR-mutated NSCLC develop resistance and treatment options on or after disease progression are limited. Phase III clinical trial data, using agents with broad antitumour activity, have demonstrated the potential to extend the clinical benefit of 1L osimertinib and delay the onset of resistance. Dato-DXd, an antibody-drug conjugate composed of a humanized anti-TROP2 monoclonal antibody conjugated to a potent topoisomerase I inhibitor, has demonstrated efficacy as monotherapy in NSCLC in TROPION-Lung01, including in patients with EGFR-mutated advanced NSCLC. TROPION-Lung14 is evaluating the efficacy and safety of osimertinib ± Dato-DXd as 1L therapy in patients with EGFR-mutated LA/M NSCLC.

Trial design: TROPION-Lung14 (NCT06350097) is an ongoing phase III, open-label, multicentre, randomised study. The study is enrolling patients (aged ≥ 18 years) with histologically or cytologically confirmed stage IIIB/IIIC or IV non-squamous, EGFR-mutated (exon 19 deletion or L858R) NSCLC, no prior EGFR tyrosine kinase inhibitor or other systemic therapy for stage IIIB/IIIC or IV disease, at least one measurable lesion per RECIST 1.1, and WHO performance status (PS) of 0 or 1. Prior to the randomised study period, ~ 20 patients will receive osimertinib + Dato-DXd in a non-randomised single-arm safety run-in. Following safety run-in, ~ 562 patients will be randomised 1:1 to osimertinib (80 mg orally [PO] QD) or osimertinib (80 mg PO QD) + Dato-DXd (6 mg/kg IV Q3W). Patients will be stratified by EGFR mutation type (Ex19Del vs L858R), WHO PS (0 vs 1) and central nervous system (CNS) metastasis status (yes vs no). Treatment will continue until RECIST v1.1-defined progression or unacceptable toxicity. The primary study endpoint is progression-free survival (PFS) assessed by blinded independent central review. Overall survival is a key secondary endpoint; other secondary endpoints include PFS by investigator, objective response rate, duration of response, PFS2, safety, pharmacokinetics and immunogenicity. Enrolment is ongoing.

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TROPION-Lung15: A phase III study of datopotamab deruxtecán (Dato-DXd) $\pm$ osimertinib vs platinum doublet chemotherapy in patients with EGFR-mutated locally advanced or metastatic non-small cell lung cancer (NSCLC) and disease progression on prior osimertinib

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Background: Disease progression on osimertinib is common and can be due to multiple different resistance mechanisms. After progression, osimertinib in combination with therapies with broad antitumour activity, such antibody-drug conjugates (ADC), have the potential to overcome osimertinib resistance. Dato-DXd is an ADC composed of a humanised anti-TROP2 monoclonal antibody conjugated to a potent topoisomerase I inhibitor via a plasma-stable linker. The efficacy of Dato-DXd monotherapy in NSCLC has been demonstrated in several studies, including in patients (pts) with previously-treated EGFR-mutated (EGFRm) advanced NSCLC. TROPION-Lung15 is evaluating Dato-DXd $\pm$ osimertinib vs platinum doublet chemotherapy (CT) in pts with EGFRm advanced NSCLC and disease progression on prior osimertinib.

Trial design: TROPION-Lung15 (NCT06417814) is an ongoing phase III, open-label, sponsor-blind, multicentre, randomised trial. The study will enrol ~ 630 pts aged ≥ 18 years with histologically/cytologically confirmed non-squamous NSCLC, documented EGFR TKI sensitive mutations, radiologic progression on prior osimertinib monotherapy (advEJ, locally advanced or metastatic disease), ≤ 2 prior lines of EGFR TKIs, ≥ 1 measurable lesion per RECIST v1.1, and ECOG PS 0 or 1. Pts will be randomised 1:1:1 to Dato-DXd (6 mg/kg IV Q3W), Dato-DXd (6 mg/kg IV Q3W) + osimertinib (80 mg PO QD), or platinum